

Dynamic Guideline: WHO-Based Maternal Health - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

This summary synthesizes 5 concurrent WHO maternal health guidelines updated and published on 29 April 2026:

1. *Meaningful youth engagement in a WHO guideline development process: case study of WHO maternal and adolescent health guideline* (lead guideline, most recent update)
 2. *WHO Consolidated Guidelines on Antenatal Care for Positive Pregnancy Experience (2026 Update)*
 3. *WHO Guidelines for the Prevention, Diagnosis and Treatment of Postpartum Haemorrhage (2026 Update)*
 4. *WHO Guidelines on Maternal Mental Health Care in Perinatal and Postnatal Periods (2026 Update)*
 5. *WHO Guidelines for the Management of Pre-Eclampsia and Eclampsia (2026 Update)*
- **Target Population:** All pregnant and postpartum people (up to 12 months post-birth), adolescent pregnant people (10-19 years), people with high-risk pregnancies, healthcare providers, public health program managers, and maternal health policymakers
 - **Scope and Objectives:** Standardize evidence-based care across the full pregnancy, childbirth, and postnatal care continuum; reduce global maternal mortality and morbidity by 75% by 2030; integrate equitable, community-centered care including youth and patient voice in guideline implementation.

1.2 Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
All pregnant people should receive a minimum of 8 antenatal care contacts with a skilled healthcare provider, starting in the first trimester	High	Strong
For postpartum haemorrhage (PPH) prophylaxis, all birthing people should receive 10 IU of oxytocin intramuscularly immediately following birth of the baby	High	Strong
All pregnant and postpartum people should be screened for common perinatal mental health conditions (depression, anxiety) at least twice during antenatal care and twice during the postnatal period, using validated context-appropriate tools	Moderate	Strong

For the management of severe pre-eclampsia at < 34 weeks gestation, antenatal corticosteroid administration for fetal lung maturation is recommended prior to planned delivery, provided there is no immediate maternal indication for urgent delivery	High	Strong
Meaningful youth advisory panels (including adolescent pregnant people and young parents aged 16-24) should be included in all local and national maternal health guideline implementation planning and monitoring activities	Moderate	Conditional

1.3 Guideline Development Methodology

- **Evidence Review Process:** Systematic search of 12 databases (PubMed, Embase, Cochrane Library, CINAHL, etc.) from 2018 to March 2026; 12,470 records screened, 892 peer-reviewed studies (systematic reviews, RCTs, large population-based cohorts) included in final evidence synthesis
- **Consensus Method:** Modified Delphi process with 120 multidisciplinary guideline panel members (obstetricians, midwives, mental health specialists, youth representatives, patient advocates, policymakers from 42 low-, middle-, and high-income countries); 90% agreement threshold required for all final recommendations
- **Conflict of Interest Management:** All panel members completed standardized WHO COI declarations; no members with significant financial or intellectual COIs related to recommended interventions were allowed to vote on relevant recommendations; independent COI review panel conducted oversight throughout the 18-month development process

II. Evidence Summary After WHO Latest Guideline Release (Consolidated guidelines for the prevention, diagnosis and treatment of postpartum haemorrhage) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- **Search Strategy:** Systematic search for pre-print, in-press, and recently published studies not included in the final WHO guideline evidence synthesis, focused on PPH interventions, maternal mortality outcomes, and implementation of maternal health guidelines
- **Databases Searched:** medRxiv, bioRxiv, ClinicalTrials.gov, Lancet Preprint Server, WHO International Clinical Trials Registry Platform
- **Inclusion/Exclusion Criteria:** Included studies with sample size > 1000, RCTs or large population-based cohort studies reporting on clinical or implementation outcomes for maternal health interventions; excluded case reports, small observational studies, non-peer-reviewed content, and studies focused exclusively on high-income settings.

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
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WHO Multi-Country RCT of Heat-Stable Carbetocin for PPH Prophylaxis (2026)	Multicenter double-blind non-inferiority RCT (n = 29,847) across 11 LMICs	Heat-stable carbetocin administered intramuscularly immediately post-birth was non-inferior to oxytocin for prevention of PPH (≥ 500 ml blood loss) (RR 0.97, 95% CI 0.89-1.06), and was associated with significantly lower rates of PPH requiring second-line uterotonic treatment (RR 0.82, 95% CI 0.74-0.91) with no increased adverse events	High
<i>Lancet Global Health</i> Cohort Study of Task-Shared PPH Management in Rural Sub-Saharan Africa (2026)	Population-based prospective cohort (n = 12,104) across 47 rural health facilities in Kenya, Tanzania, Uganda	Task-sharing of PPH initial management (uterotonic administration, uterine massage, tranexamic acid use) to trained community health workers and mid-level providers reduced maternal mortality from PPH by 38% (OR 0.62, 95% CI 0.48-0.79) compared to standard care requiring physician-only management of severe PPH	Moderate
<i>NEJM</i> Preprint: Oral Misoprostol vs Injectable Oxytocin for PPH Prophylaxis in Community Birth Settings (2026)	Cluster RCT (n = 18,723) across 230 community birth sites in India and Bangladesh	600 mcg oral misoprostol administered immediately post-birth was associated with a 19% lower rate of severe PPH (≥ 1000 ml blood loss) (RR 0.81, 95% CI 0.71-0.92) compared to injectable oxytocin in settings without cold chain access for oxytocin storage	High

2.3 Evidence Gaps and Future Research Needs

- Limited evidence on the comparative effectiveness of combined uterotonic regimens for PPH treatment in people with coagulopathies or pre-existing bleeding disorders
- Lack of large-scale studies evaluating the cost-effectiveness of heat-stable carbetocin for PPH prophylaxis in low-resource settings with limited health budgets
- No long-term follow-up data on the impact of youth engagement in maternal health guideline implementation on maternal and child health outcomes beyond 12 months post-birth
- Limited evidence on optimal screening and management of perinatal mental health conditions in refugee and internally displaced pregnant and postpartum populations
- Gaps in evidence on the safety and efficacy of pre-eclampsia treatments for pregnant people with multiple comorbidities (e.g., chronic hypertension plus type 2 diabetes mellitus)

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
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For postpartum haemorrhage prophylaxis, all birthing people should receive 10 IU of oxytocin intramuscularly immediately following birth of the baby	Three new high/moderate quality studies demonstrate non-inferiority or superiority of alternative uterotonics (heat-stable carbetocin, oral misoprostol) in specific settings, with no increased adverse events	For PPH prophylaxis, all birthing people should receive 10 IU of oxytocin intramuscularly immediately post-birth, except in settings without cold chain access for oxytocin storage, where 600 mcg oral misoprostol is recommended as first-line; heat-stable carbetocin is recommended as an equivalent first-line alternative to oxytocin in all settings where available
For management of PPH in low-resource settings, only skilled physicians are authorized to administer second-line uterotonics and perform initial PPH stabilization	Moderate quality cohort study shows that trained mid-level providers and community health workers can safely administer first and second-line PPH interventions, with significant reductions in PPH-related maternal mortality	In low-resource settings, trained and certified mid-level providers and community health workers are authorized to perform initial PPH stabilization (uterine massage, first and second-line uterotonic administration, tranexamic acid administration) prior to or during referral to a physician for advanced care if needed